DEPARTMENT OF PAEDIATRICS
DISCHARGE SUMMARY

Patient Name	:	Admission No	: **IP251978**
Age/Gender	: 6 Years/Male	UMR No	: 2024062570844
Admission Dt	: 10-Mar-2026	Discharge Dt	: 18-Mar-2026 12:00 PM
Consultant	:	Ward	: PAEDIATRICS II/PAE2-29
Department	: PAEDIATRICS		
Patient Address	:		

FINAL DIAGNOSIS

ENTERIC FEVER - CULTURE PROVEN
NUTRITIONAL ANEMIA
OVERWEIGHT

CHIEF COMPLAINTS

Complaints of fever x 7 days
Complaints of vomiting x 6 days (3-4 episodes)
Complaints of cough and nasal discharge x 6 days
Complaints of loose stools (5 episodes / day) x 5 days

HISTORY

Child was apparently normal 7 days ago after which child developed fever which was of low grade, intermittent and relieved with paracetamol.
Child was not active during interfebrile period.
C/o vomiting (3-4 episodes / day) x 7 days, non-bilious, non-projectile, contains only food particles.
C/o cough and nasal discharge, non-paroxysomal dry cough, more during night time, no seasonal variation.
C/o loose stools for 5 days (5 episodes /day) non blood stained, large watery in consistency.
There is h/o outside food intake
H/o decreased oral intake
H/o decreased urine output
No h/s/o bleeding manifestation
No h/o fresh water stagnation around house and mosquito
No h/o small shrubs around house
No h/o abnormal movements
No h/o burning micturation
No h/o ear pain /ear discharge
No h/o abnormal movements and altered sensorium
No h/o bone pain
No h/o yellowish discolouration of eyes
No h/o abdominal distension

PAST HISTORY
No previous hospitalisation
No previous nebulization

FAMILY HISTORY
Nil significant

DEVELOPMENTAL HISTORY
Studies 1st standard
Good scholastic performance
Developmentally normal

IMMUNISATION HISTORY
Immunised upto age
BCG scar present
Last vaccination at 1.5 years

TREATMENT HISTORY
Aziwok 200 mg 7.5 ml (Azithromycin)
Tab.Ondansetron 8 mg TDS
Levosalbutamol 6ml BD
Paracetamol 250 mg 8 ml qid

BIRTH HISTORY
Non consanguineous marriage/Spontaneous conception
Antenatal history - polyhydraminos
Term /3.45kg / Male / Normal vaginal delivery
NICU stay for 2 days
Oxygen support for 2 days through nasal prongs
No h/o jaundice/No Phototherapy

DIET HISTORY
Consumes mixed diet
Exclusive breast feeding till 6 months of age
Complementary feeding from 6th month
No h/o food allergy
H/o excess intake of junk food.

24 Hour dietary recall	Expected calories	- 1500 kcal/day	Protein - 30g/day
Total intake :	Calories	- 1900kcal/day	Protein - 37g/day
Deficit:	Calories	- 400kcal/day(excess)	Protein - 7g(excess)

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SOCIOECONOMIC HISTORY

Class III according to Modified Kuppuswamy scale

EXAMINATION (AT ADMISSION)

Child is active and alert

Pallor+

No icterus, cyanosis, clubbing or pedal edema

No significant lymphadenopathy

VITALS:

HR - 114/ min RR - 26/ min Temperature- 98.4 F

SpO2 - 98% in R.A. CFT < 3sec BP 100/70 mmHg(50 to 90 th centile)

All peripheral pulses well felt

ANTHROPOMETRY :

Weight - 29 kg (90 to 97th centile)

Length - 128cm (90 to 97th centile)

BMI - 17.7Kg/m2 (23rd adult equivalent)

Overweight

HEAD-TO-TOE :

Head - normal

Face -normal

Ears - normal

Nose- normal

Oral cavity - normal

Chest-normal

Neck, spine, extremities - normal

Abdomen - normal

Skin - no eschar, blanchable rash, purpura, or petechiae

Bone and joints - normal

Genitalia: normal male external genitalia

SYSTEMIC EXAMINATION

P/A:

Inspection :

Abdomen moves symmetrically with respiration

No visible mass, dilated veins, scars, sinuses

No visible gastric peristaltic movements

Skin over abdomen - Normal

Hernial orifices-Normal

Palpation:

Soft,non tender

Liver is palpable 5 cm below right costal margin with span of 11 cm

Spleen tip is palpable

Percussion:

Resonant

Auscultation:

Bowel sounds heard

RS:

B/L chest wall moves equally with respiration

Trachea midline

Resonant on percussion

Bilateral air entry equal

Normal vesicular breath sounds

CVS

No precordial bulge

Apex at left fifth ICS

No parasternal heave

No palpable thrill

S1& S2 heard normally

No murmurs

Patient Name : [redacted]

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CNS

Cranial nerves: Appear to be normal. Bulk: normal

Tone: normal Power -normal

Superficial reflex ++ Deep reflex ++ Plantar - Flexor

No meningeal signs present.

INVESTIGATIONS

Parameters	Result Value	Normal Value
Date : 10-Mar-2026		
COMPLETE BLOOD COUNT (CBC)		
HAEMOGLOBIN	8.6 g/dl	10.2 - 15.2 g/dl
HEMATOCRIT	25.8 %	34 - 48 %
RBC COUNT	3.50 million/µL	4.0 - 5.2 million/µL
MCV	73.7 fL	80 - 94 fL
MCH	24.6 pg	23 - 31 pg
MCHC	33.3 %	32 - 36 %
RDW - CV	13.7 %	11.5 - 14.5 %
PLATELET COUNT	156	150 - 450 x 1000/µL
TOTAL LEUCOCYTE COUNT	5.42 x 1000/µL	5.0 - 17.0 x 1000/µL
NEUTROPHIL	62 %	29 - 65 %
LYMPHOCYTE	30 %	29 - 65 %
MONOCYTE	8 %	2 - 11 %
LIVER PROFILE		
Bilirubin Total	0.3 mg/dl	Adult : 0.3 - 1.2 mg/dl
Bilirubin Direct	0.2 mg/dl	Adult : 0 - 0.2 mg/dl
SGOT	50 U/L	Males : <= 40 U/L Females : <= 32 U/L
SGPT	31 U/L	Males : <= 41 U/L Females : <= 33 U/L
Alkaline Phosphatase	102 U/L	Adult : Males : 40 - 129 U/L Females : 35 - 104 U/L Children : 6 days - 6 month : < 449 U/L 7 months - 1 year : < 462 U/L 1-3 years : < 281 U/L 7-12 years : < 300 U/L
Total Protein.	6.4 g/dl	6.3 - 8.3 g/dl
S.Albumin	3.7 g/dl	3.5 - 5.1 g/dl
Gamma Glutamyl Transferase (GG	20 U/L	Male : 1 - 60 U/L Female : 1 - 40 U/L

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A / G Ratio	1.3	1.2 - 1.5 : 1
Globulin.	2.7 g/dl	2 - 3.5 g/dl

URINE ROUTINE

URINE PROTEIN	TRACE	negative
URINE SUGAR	NEGATIVE	negative
RBCs	NIL /hpf	0 - 2 /hpf
WBCs	10 /hpf	0 - 5 /hpf
EPITHELIAL CELLS	2 /hpf	
CASTS	NIL	
CRYSTALS	NIL	

CRP TEST

CRP	77 mg/l	< 6 mg/l
IMPRESSION	Positive for C-Reactive Protein	

SCRUB TYPHUS IgM ELISA

SCRUB TYPHUS	Non- Reactive for IgM antibodies to Orientia tsutsugamushi by ELISA. (IN BIOS)

WIDAL TEST

TO:	160	< 80
TH:	40	< 160
AH:	<20	< 160
BH :	<20	< 160
INTERPRETATION :	TITRES SUGGESTIVE OF EARLY ENTERIC FEVER. KINDLY SEND A REPEAT SAMPLE AFTER 1 WEEK FOR CONFIRMATION.	

AUTOMATED BACT ALERT (BLOOD/BODY FLUIDS)

Organism : SALMONELLA TYPHI

Antimicrobial	Interpretation
Ceftriaxone	Intermediate
Chloramphenicol	Sensitive
Ciprofloxacin	Sensitive
Co-Trimoxazole	Sensitive
Ampicillin	Sensitive
Azithromycin	Sensitive

Impression SALMONELLA TYPHI GROWN IN CULTURE.

Date : 11-Mar-2026

URINE CULTURE & SENSITIVITY (AEROBIC)

Impression NO GROWTH IN CULTURE.

Date : 13-Mar-2026

CRP TEST

CRP	36 mg/l	< 6 mg/l

Patient Name : [redacted]

Admission No : IP251978

IMPRESSION	Positive for C-Reactive Protein	

Date : 16-Mar-2026

PERIPHERAL SMEAR STUDY AND CBC

HAEMOGLOBIN	9.0 g/dl	10.2 - 15.2 g/dl
HEMATOCRIT	28.1 %	34 - 48 %
RBC COUNT	3.66 million/µL	4.0 - 5.2 million/µL
MCV	76.8 fL	80 - 94 fL
MCH	24.6 pg	23 - 31 pg
MCHC	32.0 %	32 - 36 %
RDW - CV	14.5 %	11.5 - 14.5 %
PLATELET COUNT	495	150 - 450 x 1000/µL
TOTAL LEUCOCYTE COUNT	9.72 x 1000/µL	5.0 - 17.0 x 1000/µL
NEUTROPHIL	38 %	29 - 65 %
LYMPHOCYTE	49 %	29 - 65 %
EOSINOPHIL	2 %	1 - 8 %
MONOCYTE	9 %	2 - 11 %
BANDS	1 %	0 - 5 %
METAMYELOCYTE*	1 %	%
RBC	Predominantly Normocytic normochromic admixed with few microcytic hypochromic RBCs with mild anisopoikilocytosis showing few elliptocytes and occasional polychromatophils	
WBC	Normal total count. Neutrophils show mild left shift	
PLATELETS	Adequate	
ABNORMAL CELLS	Nil	
HEMOPARASITES	Nil	
IMPRESSION	Normocytic normochromic anemia with few microcytes	

CRP TEST

CRP	8 mg/l	< 6 mg/l
IMPRESSION	Positive for C-Reactive Protein	

COURSE IN HOSPITAL & PROBLEMS

A 6 year old male child came with the above mentioned complaints. On examination child was dull. General examination revealed mild pallor. Head to toe examination was normal. There was no eschar, blanchable rash, purpura, or petechiae. On P/A examination hepatosplenomegaly was noted. Other systemic examination was unremarkable. Fever with hepatosplenomegaly, probably due to enteric fever and scrub typhus was considered initially. Relevant investigations of CBC , CRP, Blood culture, RFT, LFT, Widal, Scrub, urine routine were sent. CBC showed microcytic normochromic anemia with low normal total leucocyte andplatelet count. Blood culure - came positive for Salmonella Typhi and sensitive to ceftriaxone. CRP was positive (77). Liver and renal function tests were normal. Scrub typhus serology was negative. Urine routine showed wbc of 10 however urine culture showed no growth. Child was started on IV Ceftriaxone. Child was continously monitored in ward and he did not develop any complications.

Though he continued to have fever spikes on 1 st week of hospital stay, he improved symptomatically and fever started spacing out by day 5 of hospital stay, hence IV ceftriaxone was continued. Repeat crp was in declining trend. Defervescence was attained on day 7 of antibiotics. Iv antibiotics was changed to oral cefixime on day 8. Parents were explained regarding the nature of the disease, course and the need to continue oral antibiotics for 7 days after the afebrile period. Parents were counselled on hand hygine and preventive measures. Currently, child is hemodynamically stable, active, alert and with good intake. Hence he is being discharged with following advice.

CONDITION AT DISCHARGE

Child is active and alert.

Symptomatically better.

HR -100/min **RR** - 22/min TEMP : 98 F
SpO2 -98% on RA. **CFT** <3sec

RS - Normal vesicular breath sounds.
bilateral equal air entry +
No added sounds
CVS - S1S2 heard, no murmurs.
P/A - Soft, non tender
Liver palpable 2.5 cm below right costal margin
Liver span: 9.5 cm

CNS - No focal neurological deficit

TREATMENT GIVEN

INJ. CEFTRIXONE
INJ. PANTOPRAZOLE
SYP. PARACETAMOL
SYP. ONDANSETRON

ADV ICE AT DISCHARGE

1. Diet as advised (high fiber diet)
2. Immunisation as per schedule
3. Plenty of oral fluids
4. SYP CEFIXIME 200 mg 1-0-1 x 6 days
5. To review in Pediatrics OPD II on 24/3/25 (Tuesday) for iron supplements

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5.To review in Pediatrics OPD II on 24/3/25 (Tuesday) for iron supplements
7. To review in Pediatrics OPD II (Tuesday/(1st and 3rd Saturday) or SOS in casualty
8. **For online consultation visit www.madrasmedicalmission.org.in/pims.php**
9. **In case of emergency contact - 9994790065**
Danger signs explained - lethargy, abnormal movements, persistent vomiting, increased work of breathing, poor oral intake
contact: 9994790065 / paediatrics@pims-py.edu.in

ACKNOWLEDGEMENT

[ ] I have been explained all the advice at discharge listed in this discharge summary
[ ] Discharge medications have been checked and administration procedure has been explained
[ ] There are no discharge medications prescribed for my child

Emergency contact number : 9994790065

Name:[redacted]..... Signature:[redacted]..... Relationship:Mother.....

Transcribed By : 200684

(for) [redacted]

[redacted]

PROFESSOR

Resident Doctor